

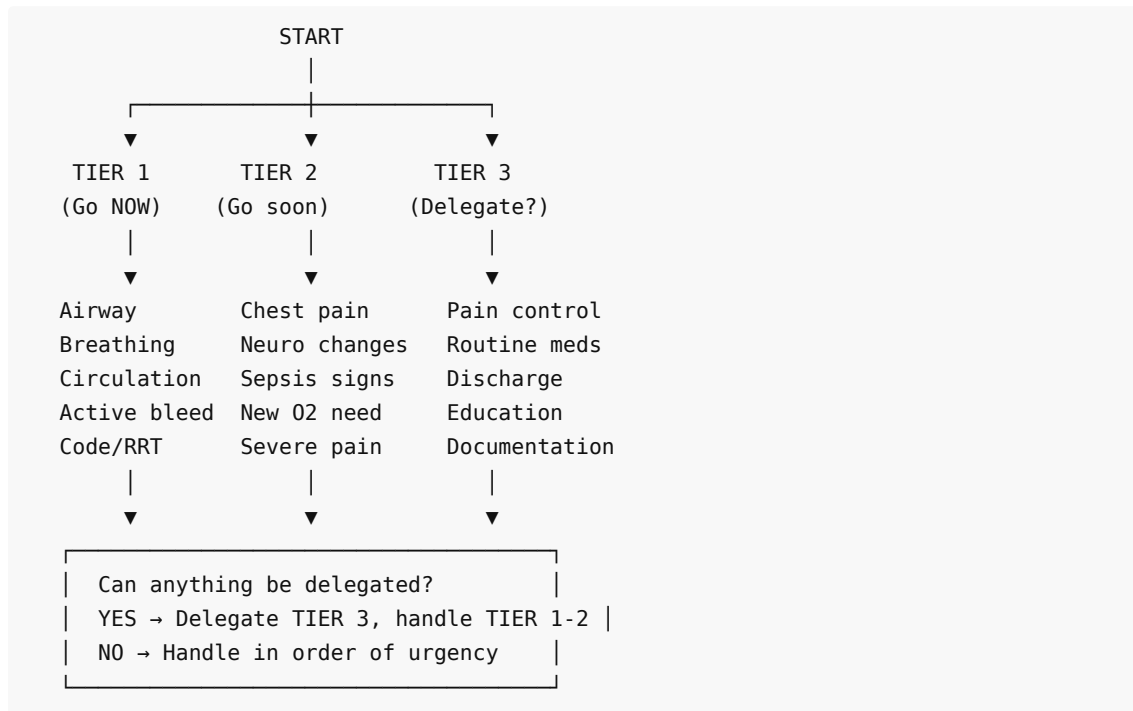
Prioritization Decision Trees

Clinical Judgment Tools for NGN NCLEX and Practice

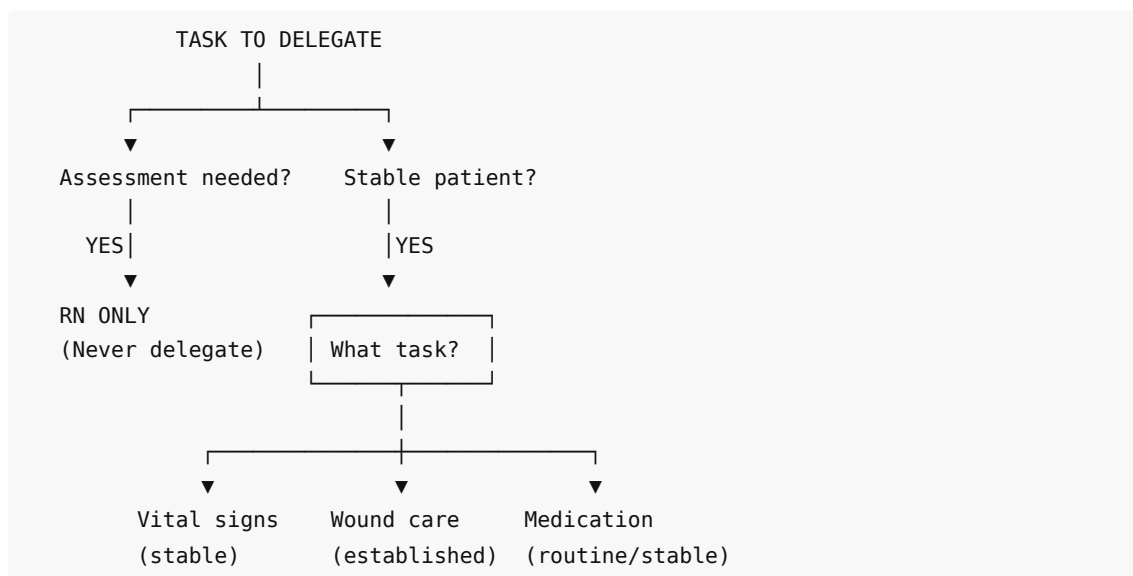
By Nnamdi Okorafor, RN

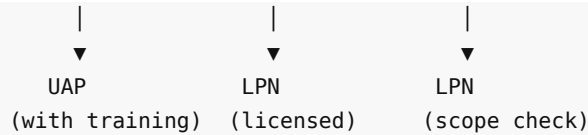
Decision Tree 1: The "Who Do You See First?" Framework

When multiple patients need you and you're the only nurse, use this tree:

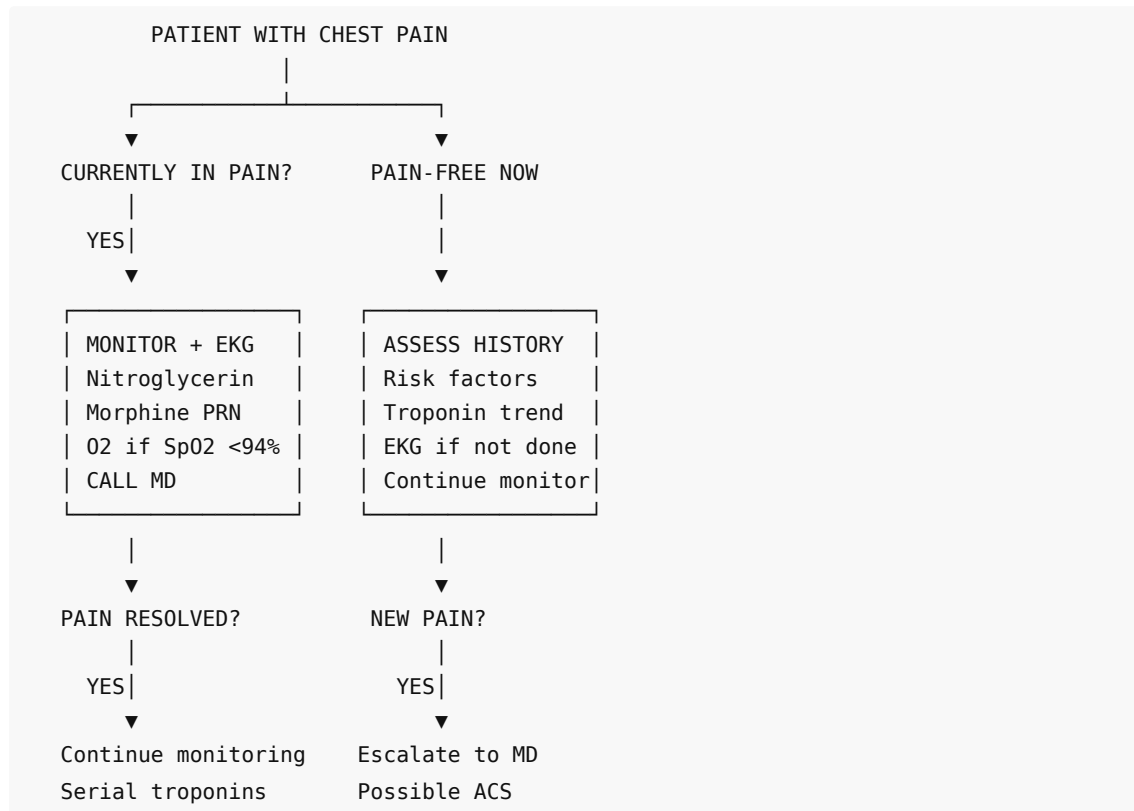


Decision Tree 2: Delegation Flowchart

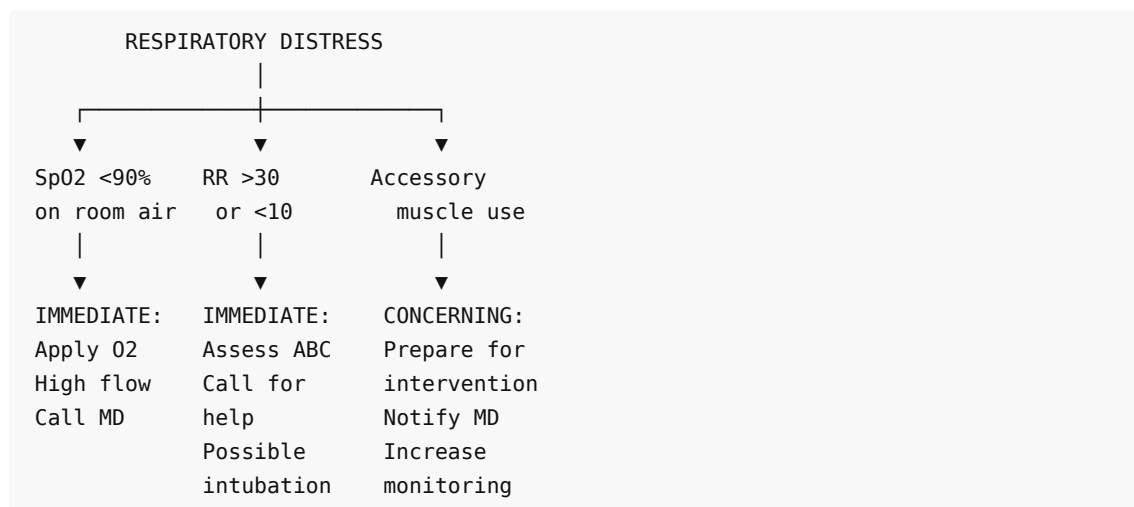




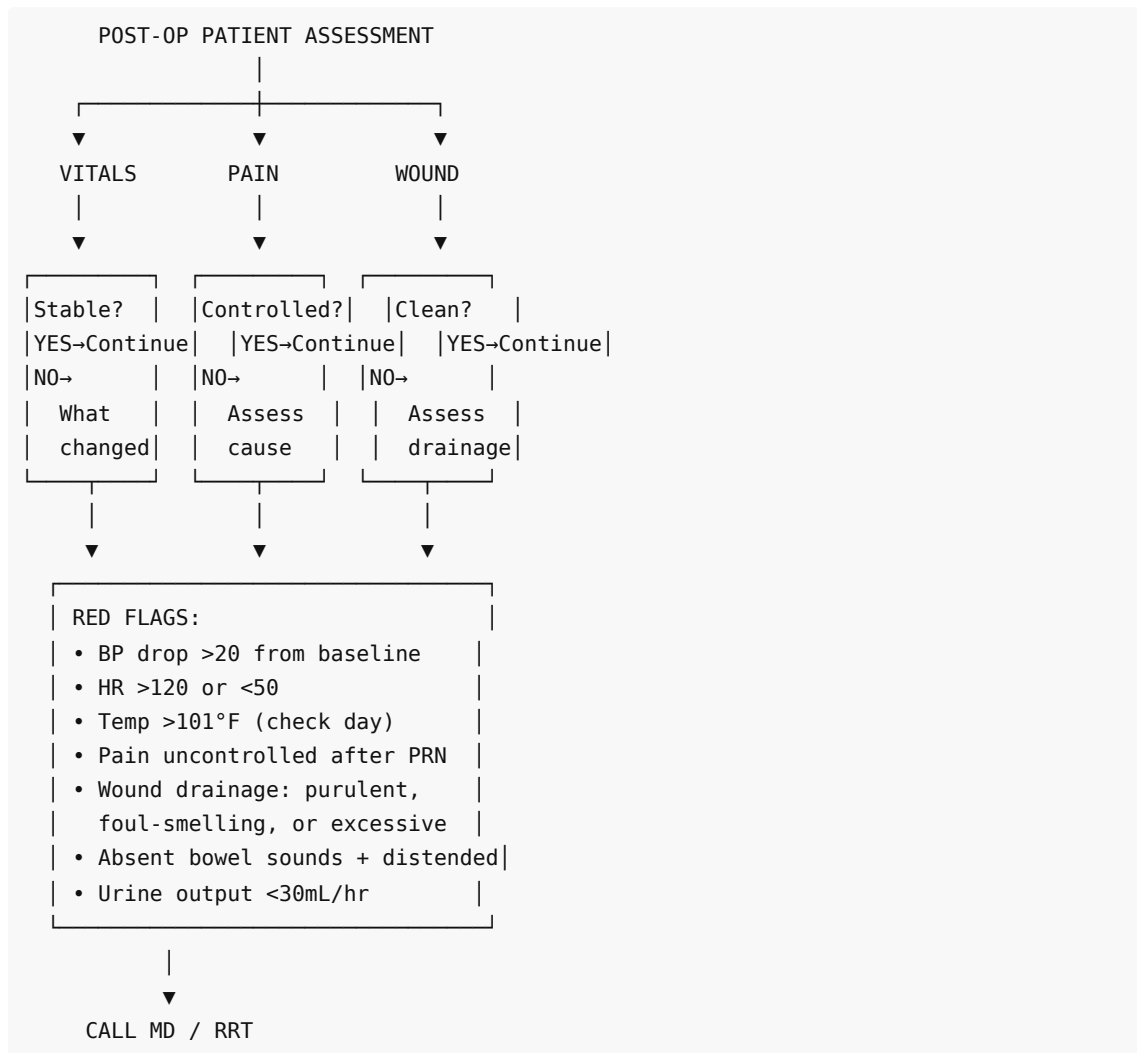
Decision Tree 3: Chest Pain Triage



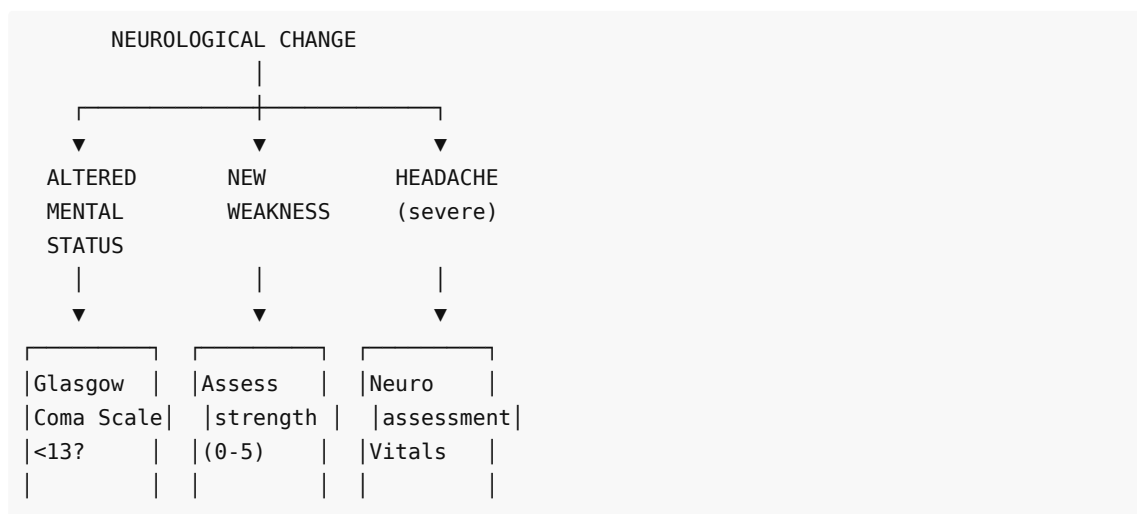
Decision Tree 4: Respiratory Distress



Decision Tree 5: Post-Op Complications



Decision Tree 6: Neurological Changes



YES→	<3?	BP?	
RRT/Code	YES→	Neck	
	Call MD	stiff?	
NO→			
Monitor	NO→	YES→	
closely	Continue	Possible	
	monitor	meningitis	
		NO→	
		Continue	
		monitor	

10 Practice Scenarios

Scenario 1: Medical-Surgical Floor

Your assignment: 4 patients

- **Room 1:** Post-op appendectomy, day 2. Pain 6/10, distended abdomen, no bowel sounds, last BM 3 days ago (pre-op).
- **Room 2:** CHF exacerbation, 82-year-old. SpO2 91% on 2L, RR 24, crackles bilaterally.
- **Room 3:** DKA, insulin drip running. Blood glucose 280, K+ 3.2, urine output 400mL last 8 hours.
- **Room 4:** Fall risk patient, mild dementia, trying to get out of bed, family at bedside.

Question: Who do you see first and why?

► Answer & Rationale

Scenario 2: Emergency Department

You're triaging. Three patients arrive simultaneously:

- **Patient A:** 34-year-old, "I think I have the flu." Temp 101.2, sore throat, otherwise healthy.
- **Patient B:** 67-year-old, "My chest feels heavy." BP 168/94, HR 96, pain 4/10, diaphoretic.
- **Patient C:** 5-year-old, fell from jungle gym. Crying, holding right arm, deformity visible.

Question: Triage order?

► Answer & Rationale

Scenario 3: Oncology Unit

You have 3 patients:

- **Patient A:** Neutropenic (ANC 400), temp 100.8°F. Last chemo 7 days ago.
- **Patient B:** Post-op Whipple procedure, day 3. NGT output suddenly increased, bilious, 800mL last 4 hours.
- **Patient C:** End-stage cancer, comfort measures only. Family requesting pain medication. Patient grimacing, moaning.

Question: Priorities?

► Answer & Rationale

Scenario 4: Code Blue Response

You're a new grad. Code called on your unit. You arrive to find:

- Patient unresponsive
- No pulse
- RN from next unit is running the code
- RT is managing airway
- Another nurse is doing compressions

Question: What do YOU do?

► Answer & Rationale

[Continue with Scenarios 5-10 in complete product...]

Quick Reference Cards

Vital Signs: When to Worry

Vital Sign	Concern Threshold	Action
SBP	<90 or >180 (symptomatic)	Notify MD
DBP	>110	Monitor, notify if symptomatic
HR	<50 or >120	Assess, notify MD
RR	<10 or >30	Check SpO2, notify MD
SpO2	<92% on room air	Apply O2, notify MD
Temp	>101°F (post-op day 1-3)	Assess for infection
Temp	>100.4°F (neutropenic)	EMERGENCY

Lab Values: Critical Values

Lab	Low Critical	High Critical	Common Cause
K+	<2.5	>6.5	DKA, renal, diuretics
Na+	<120	>160	Dehydration, SIADH
Glucose	<50	>400	Insulin, DKA
Hgb	<7	>18 (dehydration)	Bleeding, anemia
WBC	<2,000	>30,000	Infection, immunosuppression
Platelets	<20,000	-	Bleeding risk
Creatinine	-	>4	AKI, dehydration

This is an excerpt. The complete system includes 30+ scenarios with full walkthroughs.

Get the complete Clinical Judgment Mastery System at obiomacare.com